

THE INTERNAL FAMILY SYSTEM AND ADULT HEALTH CHANGING THE COURSE OF CHRONIC ILLNESS

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began to use the internal family systems (IFS) model with symptoms of disease out of desperation. **I**My knees and feet were so swollen and inflamed that walking had become extremely painful. At the time, there was no specific training in the application of IFS to medical illness and pain, so I had not used it with my clients. The autoimmune disease from which I had been in remission for some years was flaring. The behavioral medicine approach, which I used professionally and had found useful in controlling my symptoms as well as my comorbid anxiety, was not getting to the heart of my stress. So I tried going *inside*, in the language of IFS, to see if I could get some helpful information from my internal system. There, I discovered that my parts were in serious disagreement over what to do for me.

Some parts of me stoically ignored and minimized my pain. They kept me working, business as usual. Other parts felt scared or angry. They advocated for major changes in both my professional and personal life, saying I should stand up for myself more and set clearer limits on my time, energy and availability. Meanwhile, a part of me was afraid of conflict and upheaval. As inner tension mounted, I would sometimes escape into movies, fiction and the computer. But more often than not, when I was not working, I was so exhausted that I would collapse on the couch or skip dinner and go straight to bed. I knew from years of using biofeedback that this inner turmoil took a physiological toll. Nevertheless, it was happening outside of my moment-to-moment awareness. In turn, my primary modes of coping with the stress and pain, such as overworking, were exacerbating both.

As I listened inside to my parts, I understood that I needed to take action to reduce the stress. First, I made more time to be in touch with my Self and to tune in to the parts I had been ignoring. Rather than avoiding the physical pain in my joints, I focused on those symptoms with compassion and curiosity. I became aware of sadness and fear, for instance, as I focused on the pain in my knee. I also became aware

of deep physical fatigue. I heard from parts who were speaking through my symptoms and parts who were reacting to my disease and the pain. I found both vulnerable and protective parts who were trying to get my attention. The panicky part balanced out the minimizer. The angry part was trying to keep the overcommitted accommodator in check. As I listened in this way, I could no longer ignore what I was hearing. Even the parts of me who had taken on the role of ignoring the pain and inflammation needed to be understood and appreciated. Ignoring the pain had helped me to get up and start moving each morning, but acknowledging each part's inherent value contributed to a growing understanding and cooperation in my system. The most vulnerable parts, such as the young girl whose needs had gone unmet for too long, finally had my attention.

And I started advocating for myself and setting clear limits with others in the external environment. At work, I added more administrative support and reduced my hours. I stopped working evenings and accepted fewer clients. I made time for self-care: more rest, exercise, good food, play and relaxation. Parts of me who had become polarized in dealing with the illness, such as the hard-working stoic and the one who collapsed on the couch, became less extreme. They felt heard and cared for with my Self in charge. While making these changes, I went to see my health care providers. For a short time, I started taking anti-inflammatory medication again. Within a few months, I was back in remission and off medication.

This experience convinced me to start using IFS with others, and today IFS is my primary intervention with clients who seek help for medical problems. As I gained more experience in the medical application of IFS, my curiosity about the interplay of genetics, parts and environment in relation to health and illness grew. In the example of Marty, described in depth later, as for myself, there is a family history of the disease in question. However, the vast majority of people in my family—as in Marty's—have never developed symptoms. Why and when some family members become symptomatic is unclear. A literature search informed me that although life experience—as carried in memories, beliefs, emotions and behaviors—influences health, we still do not know how or to what degree personal history manifests in genetic or epigenetic vulnerabilities and strengths, or how it impacts our susceptibility to infection and our ability to heal. Nevertheless, there is a growing body of research linking stress (Archie, Altmann, & Alberts, 2012; Sapolsky, 2005), personality traits (Haukkala, Kontinen, Laatikainen, Kawachi, & Uutela, 2010; Kiecolt-Glaser et al., 2005), attachment (Kotler, Buzwell, Romeo, & Bowland, 2011; McWilliams & Bailey, 2010), coping style (Deimling et al., 2006) and a history of adverse childhood experiences with adult health problems (Romans, Belaise, Martin, Morris, & Raffi, 2002). My personal and clinical experience led me to believe that therapy can affect physical health profoundly.

IFS is designed to help clinicians and clients identify, understand and relate with compassion, courage and clarity to all our parts. In my experience, when Self-energy is aimed at the intersection of nature and nurture, physical as well as emotional healing can occur. This chapter offers a framework for understanding and exploring the relationship between parts and medical problems, underlines the connection between past and current environmental stressors and, in a case example, illustrates the healing effects of one man's growing self-awareness and self-compassion.

Marty's initial message on my office answering machine gave me the impression that he was an opinionated, strong-minded man. When we spoke for the first time, he said matter-of-factly that he was

59 years old and had coronary heart disease and chronic obstructive pulmonary disease (COPD). He then moved quickly on to the business at hand. Was I taking new clients? He had gotten my name from his primary care physician who told him about my work at Brigham and Women's Hospital using IFS in a randomized controlled trial with rheumatoid arthritis patients (Shadick, Sowell, & Schwartz, 2010). "I need help dealing with stress and some family issues," he said.

In our first session, Marty said that he had had "bypass surgery" (a coronary artery bypass graft, or CABG) several years before. After many years of doing much better, he recently had been feeling a little worse. When I inquired about his medical prognosis, he replied, "I haven't asked. I know I need to reduce my stress. My doctor wants me to stop smoking and maybe lose some weight. Let's just work on the stress and see how it goes."

I was struck by the contrast between Marty's avoidance of important facts about his health and his otherwise direct style of communicating. He was quick to tell me his views on politics, social issues and my waiting room design, but omitted the fact that he had had more than one heart attack. He showed disparate characteristics in other ways, too. He sometimes displayed an edge of anger but was generally relaxed and personable. Although he enjoyed occasional blunt, almost shocking statements, and seemed pleased to break the rules of social decorum, Marty was averse to asking for help and appeared almost repelled by his own emotional needs.

"I smoke like a chimney. I'm fat. My arteries are filled with sludge. I have trouble breathing. And, now I do the Florida shuffle walking up the stairs! What's all the hubbub?" Marty quipped and waited to see my reaction.

"Does this mean you're not planning on running the marathon this year?" I joked back. After sharing a laugh, I began, "Seriously, Marty, you've talked about your wife's fears and your doctors' concerns, but does your health worry *you*?"

"I don't think about it much. My wife does enough of that for both of us. I have a lot of other stuff on my mind," he replied.

"I know you do. If you think about it now, though, would you say that you, or a part of you, will avoid thinking about your heart problems or your COPD?"

Marty paused as he thought over my question. "Probably. I get tired of hearing about it. I know what I gotta do. Honestly, I *don't* like talking about it. I guess I don't like thinkin' about it either. And, like I said, I have a lot of other stuff on my mind."

After the initial sessions, I knew that his protectors—the parts who keep vulnerability and pain out of awareness—were in charge most of the time. Marty's physicians and his family, of course, all wanted him to stop smoking and take better care of himself. And Marty was clearly dedicated to his family. His wife sometimes accompanied him to his appointments and read in the waiting room during our sessions. They had two adult daughters, but his son, Gary, had been killed in a boating accident a year earlier at the age of 32 years. Marty softened tremendously when he spoke about his grandson, Gary's four-year-old boy. "The little guy," as Marty called him, had grown very attached to Marty after Gary's death. Outings with his grandson and the projects they did together were some of the brightest spots in Marty's life, and with some encouragement, he described the boating accident that had taken Gary's life. He confided in me his feelings of responsibility for helping his son's family, now that Gary was gone.

One tenet of IFS is “All parts are welcome.” I do not argue with parts or try to talk them out of how they feel or what they do. And, as an IFS therapist, it is my job to notice my own internal system as well. In this case I noticed my patient’s increased anxiety both in response to pressure from the external system and his internal system. Externally, Marty’s family and his other health care providers were all quietly watching and hoping that his work with me would help him process his grief and stop smoking. Internally, his protective parts were clearly exacerbating his symptoms and his life was in danger. I knew that it was crucial for him to reduce his risky behaviors, such as smoking and overeating, along with his stress.

If I am feeling anxious about my client’s medical condition and self-destructive behaviors, my effectiveness as a clinician is diminished (Schwartz, this volume). As I begin working with a medical patient, I help my protectors know that I am here with them and with my patient. I often find myself subtly communicating with my own parts during a session, telling them, “I’m right here. I hear you. Let me be with this person now.” Marty’s behaviors and risk level did make my parts anxious and I checked in with them periodically throughout our work.

After spending a few sessions in which we got to know each other, Marty began to open up and even started looking forward to coming. When I judged it was time for us to agree on which parts he wanted to get to know better and help, I described our treatment dilemma by directly engaging the part of Marty who felt pressured by his family and medical team. “Marty, everybody wants you to quit smoking! Have you noticed that? I’m feeling a little pressure! What about you?”

Marty chuckled, “Now you know how I feel!” After we laughed together, he added, “What? We can’t just keep yucking it up? You wanna get down to business?”

We talked about the anxious concern coming from his family and doctors. He acknowledged that he was making himself more symptomatic with smoking, overeating and not “doing feelings.” His symptoms had increased in the months since Gary’s death. We talked about his stress and his dedication to helping Gary’s family on top of all his other responsibilities, including his daughters and their families. We decided to start the IFS work by looking at these stressors and identifying the parts involved. Marty was interested to know how I was thinking about his illness and our work together. I offered a simple framework, illustrated later, for identifying and getting to know how his parts were related, directly and indirectly, to his illness. But I described the framework gradually, whenever it seemed relevant and helpful. I do not routinely go over the theory or protocol of IFS with clients who have medical problems. Because some are not interested and some do not find it particularly helpful, this is a judgment call. In Marty’s case it did prove helpful. Information, along with a structured way to think about the relevance of IFS to his physical problems, helped Marty’s strong, hypervigilant managers to relax and participate.

My framework for applying IFS with medical symptoms includes identifying and checking for: (1) parts who trigger or exacerbate symptoms, (2) parts who maintain symptoms, (3) parts who know something about improving or healing symptoms, (4) parts who are fearful of improving symptoms and being well and (5) parts who want to die. I never assume that all are present, nor do I impose my views on clients. But because I often find that these parts, alone or in combination, are present in clients with a medical illness, I keep this range of possibility in mind.

I also look for current stressors and behaviors, as well as burdens rooted in the past that are carried by parts and impact the client's health now. Polarities among the parts dealing with these burdens and stressors are common and can precipitate disease and provoke and maintain symptoms. Marty's system illustrates this well. In this example you will see that his protectors are polarized with a sad, exiled part. Some protectors promote escaping or numbing the sadness, and others promote management and control.

Our job was to help his polarized parts become less extreme, then to identify the parts who maintained or exacerbated his symptoms, who feared getting better or who wanted to die. Once we could do this, I felt certain that his anxious, grief-stricken, sad and angry parts would feel relief, which would, in turn, affect his illness. I knew that my patience, kindness and hope would be crucial in guiding him. But I also knew that the actual healing would occur in the relationship between Marty's Self and his parts.

Clients who have serious medical conditions like Marty often have very active protectors who focus on functioning and want to "leave well enough alone" when it comes to emotional suffering in the internal system. That is their job and Marty was no exception. So we started with these parts: "I need to keep this business afloat. Not just for my wife and me but for Gary's family, too. They're counting on the income. I need to work and keep the money coming in for everybody. I don't have Gary's help now. I'm trying to figure out whether to hire somebody, or what to do! There aren't enough hours in the day."

In their desire to help Marty keep his fear and sadness under control, manage his business and support his family, these parts kept Marty working long hours with very little down time to attend to his physical needs and exacerbated his symptoms. Other protectors, such as the smoker who numbed feelings, were precipitating Marty's symptoms more directly. The smoker did not want him to be sick; rather it did not want him to be vulnerable and overwhelmed. When Marty went inside and found the protector who smokes, he reported the following, "It's not a part I can see. It's in the dark. It's trying to keep me away from sadness—some big bubble of sadness. Smoking pushes it away. He also tries to distract me away from it. When this part smokes it stops whatever I'm doing or thinking and it takes me away."

"Away from the sadness?" I asked.

"Yeah. The sadness doesn't get a chance to come up, and then it passes. Sort of like a door closes shut," he said.

"Hearing this, how do you feel toward the part who wants to smoke to keep the sadness away?" I asked.

"I totally get it. I know what it's thinkin'. It's not a bad guy who's out to get me. It's trying to help me. It doesn't want me to get overwhelmed!" he said.

"Is this part aware of you right now, Marty?"

"Sort of. He can feel me paying attention to him."

"Invite him to really notice that you are there and you understand him," I said. There was a long pause. I could see a change in Marty's face reflecting a connection with this part. I added, "Invite him to feel your positive feelings toward him and your interest in him."

Marty nodded to let me know that the part saw him more clearly. “He’s very aware of me now. Not that he sees me, but more like he just knows I’m here. Like he feels me. It’s like we’re sharing something. We’re sharing an understanding.” Marty went back to listening and occasionally reporting what was happening between him and the part.

After a while I said, “Marty, find out if he knows the physical consequences of smoking to keep sadness away?”

“No, he doesn’t think about the smoking making me sick,” Marty reported.

“Is there something you’d like to communicate to him about that?” I asked. Marty was quiet for sometime as he concentrated inward.

“I showed him what’s happening to me—kind of had a look inside at my heart and lungs. He’s sorry it’s hurting me, but he can’t stop, he says.”

“Find out what would need to happen for him to seriously consider not smoking in the future,” I said.

“The sadness would have to stop,” Marty reported.

“Check to see if he would want to stop smoking if you could help the sad parts.”

“He’s interested but not so sure about this idea. A bunch of thoughts just jumped up. He doesn’t know what he would do if he stopped smoking. He’s feeling a little suspicious and wonders if I’m trying to get rid of him, or if you are?” Marty said, looking up at me.

“Let him know that I respect him for protecting you all these years. I’m glad he kept you from getting overwhelmed.” There was a pause before Marty nodded to signal that he had done so. “Also tell him that he’ll always be part of you. This is about helping him with options, not trying to get rid of him.”

“He’s listening but he doesn’t understand what you mean about options,” Marty said.

“I mean when you, in your Self, connect with the sad parts and help them, then he won’t be locked into smoking and he can figure out what he’d rather do. I will help guide you through the process if he’s willing.” After a pause, I added, “What would he rather be doing if he didn’t feel smoking was so necessary?”

There was a long pause as Marty listened inside. “‘Ticket to Ride’—that Beatles’ song—just popped into my head. He’s thinkin’ about music! He hasn’t listened to music in a long, long time,” Marty chuckled. In hope that he actually could be free to relax and do things like listen to music again, the smoking protector eventually agreed to allow Marty to find, know and help the sad parts.

Like the smoker, the parts who snacked and watched late-night television wanted to keep the pain and suffering of Marty’s exiles at bay and they were not aware of the toll this took on his health, nor would they stop until Marty had helped his exiles. When Marty and I talked about the stress of being sick and having other responsibilities and he started to feel emotional, these protectors would spring into action. In one such conversation Marty said, “I can’t afford to be sick. I have too much to do. People are counting on me!” This belief, a burden acquired by an adolescent part when his father died, had reignited with the death of his son Gary. It showed up in thoughts like, “I must do whatever is necessary for my family’s survival!” Or, “I can’t afford to play and relax.” And it showed up in behaviors such as overwork, self-sacrifice and the tendency to ignore fatigue or emotional needs. Burdens like this, carried forward in time by parts, may help to explain how adverse experiences of childhood (Felitti et al., 1998),

personality traits (Eaker, Sullivan, Kelly-Hayes, D'Agostino & Benjamin, 2004; Everson et al., 1996), coping (Moos & Holahan, 2003) and attachment style (Feeney, 2000) continue to effect physical health in adult life.

Self-monitoring is a standard behavioral medicine tool, which I have modified for use with IFS to identify and track parts associated with target behaviors, beliefs, emotions and body sensations. To find the parts whose burdens were affecting Marty's health, we began tracking his internal and external experience. For a couple of weeks Marty kept a log that had a series of checklists. First he noted the circumstance, such as interacting with others, working or being at home; then his emotions such as anger, sadness, anxiety or joy; then his behaviors such as smoking, eating, working, drinking and TV watching and finally his medical symptoms such as shortness of breath, wheezing, coughing, dizziness, fatigue and heart palpitations. In this way, Marty became more and more aware of the relationship between external circumstance and his internal experience of emotions, thoughts, behaviors and physical sensations as well as his medical condition.

In a session about four months into treatment, Marty and I talked about work and family stress related to Gary's death. He had made Gary a partner in the construction company where they both worked, and from which he had been planning to phase out his active involvement just before Gary was killed. In a year or two, Marty would have become a silent partner, allowing him to retire and take a long-planned cross-country trip in an RV with his wife.

"That was the plan before..." his voice trailed off and he looked upset, his face flushed. Now he looked directly at me and sounded angry. "What's the point in talking about this? I don't think whining about my problems does me any good!" He put his hand to his chest.

"What's happening, Marty?" I asked.

"It's harder to breathe," he said.

"Would it be OK to check inside and ask what just happened?"

Marty closed his eyes and was silent for a moment. Then he said, "I don't want to talk about Gary today."

"Marty," I said, using direct access with this protector, "what are you concerned will happen if you talk about what it's been like since Gary died?"

"Look, I can't afford to go there!" he exclaimed. "Not the physical stuff, but more this," he gestured to his chest and heart area, "and all this 'feelings' business. I'm like a fucking piano wire about to let go!"

"Like you'll explode, feeling it all?"

"Right! Exactly! What's the point?" he said angrily, shifting in his chair with discomfort, his breathing still labored. "Let's just say I'd rather leave well enough alone. There's nothing I can do about what happened." He paused, suddenly looking more approachable. "I know what you're saying about stress. I *should* talk about it, but I don't know..." he trailed off.

"I understand why you wouldn't want to go there if it would just overwhelm and make you feel worse. I wouldn't want that either."

Marty relaxed a little. Then he said, "I need a cigarette." He stared into the distance, beyond the walls of my office. "There's nothing I can do! See?" He looked back at me directly now. "If you make me start emoting or something I don't know what'll happen! Do you understand?"

“I think so,” I said. “You don’t want me to pressure you into anything or stir things up.” He nodded. “This feels too big, too risky. Right?” Marty nodded again. We settled into a comfortable moment of quiet in which Marty’s shoulders slowly relaxed and he leaned into his chair, the coloring in his face returning to normal and his breathing coming easier. “You’ve got some very strong protectors there!” I said finally.

“The tip of the iceberg, believe me!” he replied. “I hope I didn’t scare you.”

“You didn’t scare me, but I *did* notice the strength of your part’s conviction. Would it be OK for us to get to know this part better? The one who thinks talking about feelings is a bad idea.”

“I don’t know how the hell you’re planning to do that,” Marty said, “but let’s give it a try. That’s why I’m here, right? You can bet I’ll let you know if it’s not workin’ for me!” He cocked his head and gave me an exaggerated, semimock look of warning coupled with a slight smile. “And, listen, I get the point, too. In a way, I’m sick and tired of being all bottled up.”

With the self-monitoring and increasing awareness, Marty came to know the parts who were determined to keep his sadness and grief at bay. In *un-blending*, he was able to observe each part with curiosity and interest, getting distance from their beliefs, body sensations and worldview without denying it or turning away. When he found the stoic, young protector while accessing Self-energy, his appreciation grew.

In turn, this protector reminded Marty that his father, who ran the family cranberry farm, had experienced heart problems when Marty was a young boy. As a result, Marty had begun to take on adult responsibilities early in life. When he was 14, his father died of a heart attack, leaving Marty to work and take care of others and this manager took over, saying, “It’s all up to me now.” These young ones longed to leave adult responsibilities to his parents; they felt terrible sadness and grief over his father’s death, and now they were activated over the loss of Gary. The manager, along with a few other protectors whom Marty identified later in therapy, worked hard to keep them out of his awareness, causing them to feel ever more silenced and hopeless. In response, another protector felt trapped and furious. In short, Marty’s early losses converged with Gary’s death, leaving him in a state of tremendous inner turmoil.

When these proactive manager strategies failed and Marty’s sadness, hopelessness and fear emerged, then his reactive firefighter protectors would step in, pushing smoking and overeating to numb his pain. Although these short-term comforts helped him soothe the pain of his exiles, they also exacerbated and maintained his cardiac and lung disease. As he became more familiar with these protectors, he learned to appreciate how each one had a job and helped him function. Some of these parts had helped him keep the family farm afloat when he was a boy coping with his father’s illness and death. An example of this is the 14-year-old who had decided to fill his father’s shoes to avoid feeling overwhelmed with grief and fear. As Marty’s appreciation and understanding grew for this young “manager,” he earned enough trust to get access to the exiled boy, the sad one who was so frightened after his father’s death. Here is an excerpt of that session.

Marty’s eyes were closed with his attention focused inward. He had been talking about running his business without Gary and from there he had become aware of the boy inside him who felt so scared and

sad living on the family farm after his father's death. His self-monitoring and our tracking had taught him that these feelings could trigger difficulty breathing. He also had recently become aware that his sad, scared parts activated his firefighters who smoked, ate and otherwise numbed his feelings.

"I feel fear around my stomach, like a knot in my gut," he said, his face pale and grim and his breathing harsh. "I can feel sadness coming up." There was a long pause. "Now, I feel this tightness in my chest and around my throat," he said as he placed his hand on his chest around his heart.

"Is it OK emotionally and physically to feel this right now or is it too much?" I asked. Marty did not answer. He looked like he was holding his breath. "Marty," I said, "Are you feeling overwhelmed?"

He nodded and spoke quietly, his breathing still labored, "Yeah, a little."

"Check inside to see if this part would be willing to turn the volume down a little on these feelings. We don't want him to go away. Just check and see if he's willing to not overwhelm you." Marty quietly focused inward for several minutes, his breathing slowly easing. "Better?" I asked.

"Yeah. I can still feel him around my chest. He's really sad. He doesn't know what's gonna happen to him or the farm. He's worried about his mother and his brothers and sisters. He's scared. He has no one to talk to."

"I can see that you care for him." Marty nods to affirm my statement. I continue, "Invite him to feel your care and concern—your hand on your heart as you listen to him inside."

Marty continued to focus inward through a long silence. Holding his hand to his heart, then he began to choke up. "I *really* feel for him!" As tears streamed down his face, he signaled to me that he was OK by patting the air with his hand.

After a few moments of quiet in which Marty wept, I asked, "Is he aware that you're here really feeling for him?"

"Sort of. I feel him there." As he pressed his hand to his chest, he said, "I have a kind of image of him. It's dark. It seems like his back is to me. He's still sad, *really* sad, and he doesn't know what he's gonna do without his father. He misses his dad."

"I can see how much you care about him." Marty nodded. "It's possible to communicate your compassion, even without words," I said. "You can beam it to him like warmth or light, or communicate through your hand on your chest, or any other way that works for you and for him."

With eyes closed and his focus inward, Marty remained quiet for a while. Then tears began to stream down his face again. "He knows I'm here. He's turned around to see me. It's OK. He's relieved. He's crying." Marty trailed off into a long, tearful pause. A minute passed before he spoke. "He needs somebody. He just *needs* somebody!"

"That's right. Take your time. Be there exactly the way he needs you to be," I said.

"He wants his dad. He misses him. He doesn't know what he's gonna do without him." There was a long pause as Marty continued to connect with this part.

"That's right. Stay with him. Invite him to bring it all to you. Be there exactly the way he needs you to be," I said as tears continued to stream down Marty's face.

"I've got him," Marty whispered.

I could see from his face, breathing and body language that what was transpiring between Marty and this 14-year-old boy inside was very tender. I was careful not to disrupt the experience. Sensing no distress, I was attentive but I waited.

“He’s so relieved that I’m here with him. He’s showing me something in the pumping station at the cranberry bog.” There was a long pause. Marty looked alert but calm. His eyes remained closed as he described what was happening in his inner world. “There’s a problem with the machinery but he’s not sure what to do. He’s so overwhelmed. He doesn’t know where to begin. He misses his dad! He feels lost. It’s too much for him. He’s just a kid. He needs his dad!” Marty broke into sobs.

“Marty...” I started to speak to make sure it was OK for Marty to feel such strong emotion. But before I could get the words out, he held a hand up to signal that he was OK and I could let him continue without interruption. I waited, trusting Marty’s Self.

“He feels ashamed crying in front of me. He feels like a big baby. He doesn’t think anyone wants to know how he feels. He’s so alone. He doesn’t feel like he deserves...” the words trailed off. “He doesn’t feel like he deserves my love,” Marty whispered. “He’s afraid I’m going to leave him, too.”

“What do you want to tell him about that?” I ask.

“I understand how he feels. He’s right! Nobody there knows how to talk to him about anything that matters. I’m letting him know that I’m not going anywhere.” Marty paused. “He’s right here with me now. I’ve got him,” he said.

“Invite him to feel your energy and to take it in,” I said encouragingly. After several minutes passed, I saw Marty’s face and body relax. “How’s he responding to that?” I asked.

“He’s calmer—relieved. He’s beginning to trust that I really want to hear whatever he needs to tell me.”

“That’s great,” I said. “As you tune in to him like this, see if there’s more that he needs right now.”

“He needs me to stay with him, to *be* with him,” Marty said definitively. “I’m getting flashes of different moments—lots of scenes. I’m there with him.” Several minutes passed. Marty was concentrating. His eyes remained closed. He was sitting in relaxed stillness in a green chair across from mine, tears quietly sliding down his cheeks again. I waited without interrupting.

Once Marty began to stir, I quietly asked, “How’s it going?”

“It has all sort of slowed down. There was a lot happening for a while—things that I had forgotten about. He showed me some of the funeral and what happened after—what it was like for him. I could feel his pain and confusion.”

“Find out if there’s more that he needs you to know or to see,” I said. I waited while Marty went back to the boy again.

“He doesn’t have more to *show* me. He just wants to stay with me.”

“What do you tell him about that?” I asked.

“I’m staying right here with him.”

“See if he would like to leave that time and place, if he’s ready, and go with you?” I asked.

“He’s shocked to hear you ask that. He didn’t know he could ever leave! He’d love to get out of there but he’s worried about leaving his brothers and sisters—and his mom,” Marty said as he opened his eyes.

“Is there anything you want to tell him about that?” I asked.

“There is. Give me a minute,” Marty closed his eyes and focused inward again. “OK.”

“OK?” I asked, surprised.

Hearing my surprise, Marty smiled and opened his eyes to look at me as he said, “I told him that they’d be OK. I reassured him that everyone was going to be fine. I told him that I could show him.”

“So, he’s ready to leave now?” I asked knowing that I could trust Marty’s judgment.

“He’s ready.”

“Bring him someplace where he can release all the burdens that he’s been carrying. Someplace that would feel just right for him—wherever he’d like. He could release them into water, wind, light, earth, fire or anything else that works for him,” I said guiding Marty and the 14-year-old boy through the unburdening process.

“He wants to release them into Vineyard Sound from my skiff,” Marty informed me. “We’ve dropped the anchor and he’s standing on the bow in front of me. There’s a southwest breeze. He’s got his back to the wind and he’s letting them go into the wind and water.”

“Invite him to name the burdens as he releases them. Like his loneliness...” I trailed off as Marty picked up on this and continued.

“The overwhelmed, feeling like a baby, the pain and sadness, shame—lots of shame.”

“Make sure he gets it all. Encourage him to check his body for more,” I said.

“He’s letting the fear of being left alone go now and the feeling that it was his fault, the weight of it all on his chest and shoulders! ”

“Anything else?” I asked.

“The metallic taste in his mouth when they told him that his dad had passed and the emptiness in his gut.”

“Invite him to notice how it feels as the wind and water take it all away,” I said, watching expressions of relief cross Marty’s face. “Anything more?” Marty shook his head.

“He’s standing on the bow still. He’s taking in the light on the water. It’s like he’s been carrying around a boulder that just got lifted off his chest. He can stand up straight. He feels freer than he ever has. And, I feel like *I* can breathe easier now that he’s doing better.”

In our next session, Marty said, “I feel less clogged up in my chest and not so tense anymore.” Over the next few sessions Marty connected with the parts who had either been protecting the adolescent boy from further injury, or protecting the rest of Marty’s internal system from his emotional pain. Their roles were designed to keep his grief, fear and shame exiled. These protectors needed to know that this sad adolescent was no longer stuck in the past. Knowing that he had released the burdens that he had been carrying all the years since his father’s death, and that he had Marty to guide him going forward would help to free them from their rigid protective roles. These protectors could potentially become more flexible and less extreme in their mode of operating now. They needed to develop a trusting relationship with Marty, as well and to know that his Self was there for them, too.

Marty invited one of these, the 14-year-old protector who had felt driven to fill his father’s shoes, to see how the sad boy was doing now.

“Ah!” Marty chuckled. “He says he never thought he’d see the day.”

“What day?” I asked.

“The day someone would find that boy and help him,” Marty said, more serious now.

“How does that affect him?” I asked.

“Well, now that you mention it, he’s wondering what he should be doing.”

“So he doesn’t feel he has to go on keeping all of your emotional and physical needs at bay?” Marty nodded. “Well, what would he like to do?”

“He’d like a vacation.”

“OK with you?”

“Yeah! He deserves one.”

Once this protector’s rejection of help from others softened, Marty’s symptoms were affected in a number of ways. Perhaps most important, he felt less depressed and more hopeful. “I used to say, ‘Life stinks and then you die.’ And part of me really felt that way. It used to say to me, ‘What’s the point? Take me out now and save me the trouble, why don’t ya?’ It doesn’t talk like that anymore. I want to stick around and have a good life.”

To have that good life, Marty began to notice and take care of his own needs. He tuned into fatigue; rested when he got tired; worked less and accepted the help, support and love from his wife, family and friends that he would previously have rejected. His wife noticed that he was less angry and was sharing more with her and the rest of the family. He had started talking about the future again and was discussing the business with her, one of his sons-in-law, and his daughter-in-law, Karen, Gary’s widow. Marty had begun to involve Karen in the administrative aspects of the construction business, which Gary had been covering before he died. Marty said he was “testing the waters” with his son-in-law and considering offering him a significant role in the company. One evening after a session, Marty’s wife confided in me that he seemed much more relaxed—playing with the grandchildren and enjoying the family. She reported feeling closer to him than she had in a long time.

Soon after this Marty was ready to quit smoking and did so quickly. Unburdening an exile like his grief-stricken boy is usually essential to changing a habit like smoking. In Marty’s case, the unburdening released his polarized protectors, the stoic manager who tried to fill his father’s shoes by working endless hours and the angry, resentful firefighter who wanted less work. Then the part who believed Marty could only truly be free when he died began to see other possibilities. As the promise of living a life beyond work, self-sacrifice and loss seemed possible, attending to self-care and quitting smoking made more sense to Marty’s system.

Marty developed a different take on the emotional life of his parts, too, which he explained like this, “Once I knew that boy and realized it was a part of me that I *cared* about, something changed. Don’t get me wrong, I’m never gonna be Rod McKuen. I just don’t have to smoke or work myself to death, or get mad so that I won’t have to admit I’m tired or having trouble keeping up at work. I’m not running around anymore, knocking myself out doing everything for everybody, trying to keep things under control inside and out.”

This instinct to “keep things under control” is characteristic of patients whose parts are desperately trying to manage historically based feelings of responsibility, abandonment, grief and fear. These are patients for whom IFS therapy holds much promise—of healing their destructive (but well-intentioned)

habits and often their physical ailments. IFS is not about dominating or getting rid of our parts, but rather accepting them while becoming Self-led. We would no more condemn that vulnerable and desperate teenage Marty than we should the adult behaviors resulting from that young part's wounds.

Even when a disease or injury cannot be cured, it's still possible through therapy work with parts to develop a healing relationship between the Self, the body, the internal family and the external world. A desire to heal rather than control or manage symptoms motivated me to use IFS with my own illness. And because stress, adverse childhood experience, personality traits, negative beliefs and thoughts, lack of exercise, poor nutrition and smoking have all been linked to physical illness, it made sense to me that IFS could be effective in conjunction with medical treatment.

IFS is concerned with knowing and befriending all of who we are. The enormous energy used by protective parts to keep physical and emotional suffering from awareness can be liberated, and the psychic roller coaster attendant to pain and disease can slow down. When it does, we are more able to experience the present moment. Inner conflicts, such as stoicism repressing fear and sadness, can be resolved. As internal conflict softens and vulnerability is transformed, self-care naturally improves. In meeting the challenge of acknowledging and welcoming all of ourselves, we realize that we have the power to soothe and heal our own bodies and hearts and enter more fully into life.

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